THESIS PRESENTATION

Digital Health Information Access and Reproductive Health Knowledge, Attitudes and Practices Among Adolescents in Bangladesh

Nasima Akter · Supervisor: Dr. Md. Shakhaoat Hossain , Chairman and Professor

Department of Public Health and Informatics, Jahangirnagar University

Introduction , Justification and Research Gap

Information is hard to get

Taboo, limited access and poor communication restrict adolescents' adolescents' SRH information.

Family and school have limits

Mother-daughter communication is often poor and centred on menstruation.

Digital access is unequal

Online benefits may reach mainly educated, urban and wealthier adolescents.

Gap: few Bangladeshi studies measure digital access directly alongside knowledge, attitudes and practice.

Aim: to assess how digital health information access relates to adolescent KAP.

RESEARCH QUESTION

Among adolescent students in urban and rural Dhaka and Rajbari, how is access to digital reproductive health information associated with reproductive health knowledge, attitudes and practices, and which socio-demographic factors are associated with that access?

Objectives

GENERAL OBJECTIVE

To assess the association between access to digital health information and reproductive health knowledge, attitudes and practices among adolescents in Bangladesh.

Specific objectives

1. To identify the major sources of reproductive health information among adolescents.
2. To measure the level of reproductive health knowledge.
3. To assess adolescents' attitudes toward reproductive health issues.
4. To evaluate reproductive health-related practices.
5. To determine socio-demographic and other factors associated with access to reproductive health information.

Methods

Item	Detail
Design	Quantitative cross-sectional study
Setting	Five schools and colleges in Dhaka and Rajbari
Sampling	Convenience sampling, fixed numbers per institution
Sample	250 collected, 239 analyzed (133 rural, 106 urban)
Data	Google Form on mobile phones, August to September 2026
Analysis	R: Wilcoxon and Kruskal-Wallis tests, Spearman correlation, linear regression with robust SEs, ordinal logistic regression

Multiple testing adjusted (Bonferroni, Holm)

Information sources and KAP levels

31.8%
Parents

30.5%
Schools

15.5%
Internet

14.6%
Friends

Domain	Excellent	Poor
Knowledge	51.88%	31.80%
Practice	53.97%	24.27%
Attitude	95.8% favorable, with a ceiling effect	

Main source of information; N = 239

Digital access and its link with KAP

8.97

Mean access score out of 18, a moderate level

11.5 vs 9

Median access, urban vs rural (adjusted p = 0.002), a small effect

Access with	Spearman ρ	Strength
Practice	0.698	Moderate
Knowledge	0.683	Moderate
Attitude	0.384	Weak

All correlations p < 0.001 (Holm-adjusted); N = 239

Adjusted associations

Linear regression: access and scores

Outcome	β	p
Knowledge	0.670	< 0.001
Attitude	0.390	< 0.001
Practice	0.776	< 0.001

Ordinal model: better practice category

Predictor	OR (95% CI)
Access	1.52 (1.36 to 1.71)
Knowledge	1.35 (1.18 to 1.54)
Attitude	1.01 (0.96 to 1.07)

Cross-sectional associations: they do not show that access causes better knowledge or practice.

Linear models adjusted for socio-demographic factors; N = 239

Conclusion and recommendations

Family and school were the main sources. Digital access was associated with better knowledge and practice, and less strongly with attitude.

Teach digital health literacy, including how to verify online information, in schools.

Train teachers and support parents to parents to talk about reproductive reproductive health.

Prioritize rural and low-access adolescents, with face-to-face links to services.

Limits: cross-sectional design, self-report, convenience sample (N = 239)